June 16, 2025

[CMS-0042-NC] / RIN 0938-AV68

Mehmet Oz, M.D., Administrator, Centers for Medicare & Medicaid Services
Thomas Keane, M.D., Assistant Secretary for Technology Policy and National Coordinator for Health Information Technology

Re: Request for Information; Health Technology Ecosystem
Centers for Medicare & Medicaid Services,
Department of Health and Human Services,
Attention: CMS-0042-NC,
P.O. Box 8013,
Baltimore, MD 21244-8013

Dear Administrator Oz and Dr. Keane,

Surescripts appreciates the opportunity to submit comments in response to the Request for Information issued by the Department of Health and Human Services' (HHS) Centers for Medicare & Medicaid Services (CMS) and the Assistant Secretary for Technology Policy (ASTP)/Office of the National Coordinator for Health Information Technology (ONC) (collectively, ASTP/ONC).

The Surescripts Network Alliance brings together nearly all electronic health records (EHR) vendors, pharmacy benefit managers (PBMs), pharmacies and clinicians – plus health plans, long-term and post-acute care (LTPAC) organizations, specialty pharmacy organizations, and veterinary technology vendors across the U.S. Surescripts also innovates in partnership with standards bodies and governance frameworks including Carequality, DirectTrust, HL7, the National Council for Prescription Drug Programs (NCPDP), The Sequoia Project, and the Trusted Exchange Framework and Common Agreement™ (TEFCA™).

Innovations stemming from these partnerships reached healthcare professionals and patients across the U.S. in 2024, and the following statistics are based on Surescripts *Annual Impact Report*[1]:

- 2.29 million healthcare professionals and provider organizations connected to the Surescripts network
- Virtually every insured American represented in the Surescripts Master Patient Index
- 27.2 billion health intelligence transactions exchanged
- 38 milliseconds to retrieve an accurate patient match on average
- 2.6 billion e-prescriptions filled
- 889,907 prescribers using Real-Time Prescription Benefit (RTPB).

Summary of Recommendations

- **Building upon and expanding TEFCA participation**

[1] Annual Impact Report 2024 | Surescripts

- **Harmonizing privacy regulations touching consumer data access**
- **Incentivizing HIEs to participate in TEFCA**
- **Considerations for a national provider directory**
- **Approaching identity verification using IAL2, a master patient index, and a digital identity credential such as CLEAR**
- **The importance of pharmacist services and pharmacy interoperability in Advanced Payment Models (APMs) and Value-Based Care (VBC)**

Selected Questions and Responses

PC-10 How is the Trusted Exchange Framework and Common Agreement™ (TEFCA™) currently helping to advance patient access to health information in the real world?

a. **Specific examples**: TEFCA enables patient access through the secure exchange of information. Participants' efforts to define individual access involve validation and use of security tokens to verify the identity of the individual making a request. The work TEFCA participants and the RCE are doing involves using existing identity frameworks to implement the individual access requirement across multiple networks.

b. **Suggested changes**: While the Common Agreement extends HIPAA-like protections to information gathered by individual access services, further steps are required to define these organizations and regulate them specifically to protect patients. For instance, as data moves from a health care organization to an individual access service, it falls under both HIPAA and FTC privacy rules related to health information. Although efforts have been made to align these regulatory frameworks, gaps remain and additional protections are necessary.

Two notable discrepancies between HIPAA and FTC regulations include:

> 1) breach notification processes, where HIPAA mandates a patient-centric response, but FTC guidelines are less explicit, potentially implicating the healthcare provider, their technology partner, and any involved QHINs in the event of a consumer application breach.
>
> 2) HIPAA provides stringent protections for patient data, prohibiting its use without explicit patient consent, while FTC regulations only address deceptive practices or fraudulent actions by organizations. Although these issues fall outside CMS and ASTP/ONC jurisdiction, addressing these discrepancies is essential to fostering more interactions with patient access services.

We believe the Administration can and should build upon and expand the role of TEFCA. Specifically, use cases can be expanded from patients and providers to health plans and payers. To strengthen TEFCA's role as a national health information exchange framework, CMS should consider making its own data - including encounter, claims data, and research data - available through TEFCA. The robust CMS data resources (used in compliance with applicable privacy and security requirements), would be a significant addition to TEFCA and its premier role as the exchange framework for the U.S. healthcare sector.

To further support and expand the growth of TEFCA, we encourage CMS and ASTP/ONC to consider novel ways to facilitate more stakeholders to transition to TEFCA participation. Some suggestions include: additional industry education, conditions of participation incentives, and other forms of engagement, such as through the provision of technical and financial assistance to smaller, rural and/or payer organizations.

Lastly, we encourage CMS and ASTP/ONC to broaden the TEFCA framework to include participation to other providers, including cash-pay or "concierge" practices.

PC-11. How are health information exchanges (HIEs) currently helping to advance patient access to health information in the real world?
How valuable, available, and accurate do you find the data they share to be?

Health Information Exchanges (HIEs) are creating redundant data storage and processing that can be better accommodated through the national frameworks. HIEs create redundant exchange of information and introduce overall cost into the healthcare system for EHRs and other technology services to maintain and support multiple connections to these networks.

One example of the scope and scale of connections points:

If we were to examine100 nationwide technology systems (EHRs, care management applications, etc.) connecting to approximately 30 various states and/or regions with operating HIEs, this would yield a minimum of 3,000 supported connection points for a single messaging framework (i.e., Query/Response through the IHE frameworks[2]). Add to this example the many exchange modalities supported and sometimes required, like ADT, push messaging, reporting requirements, public health, etc., and the connectivity framework reaches well into the tens of thousands of interfaces unnecessarily maintained by technology vendors and the states. At a minimum, maintenance for all these interfaces is costly - estimated at several millions of dollars - to support the same exchange purposes as those envisioned in TEFCA.

What changes would you suggest?
We suggest encouraging states to adopt TEFCA through Medicaid programs, establishing clear rules for information exchange to support Medicaid services, public health, and other state and regional activities.

c**. Are there particular examples of high-performing HIE models that you believe should be propagated across markets?**
In a fully connected environment, the HIE model becomes unnecessary. Instead, such services should prioritize utilizing the connectivity framework established by TEFCA to access data as required to support the diverse interests of the states.

d. What is the ongoing role of HIEs amidst other entities facilitating data exchange and broader frameworks
We recommend HIEs transition to technology services that leverage the data accessible through TEFCA. For instance, states have a significant need to access clinical information for their Medicaid program beneficiaries. HIE services in these states should concentrate on securing

[2] Technical Frameworks - IHE International

appropriate access to data via TEFCA and offer state-level services to enhance the understanding of the effectiveness of care delivered to state residents. Public health support at a regional level is another compelling interest; however, a centralized repository may be redundant. These use cases should instead utilize TEFCA to access real-time information from various sources and partners connected through this framework. HIEs could focus on delivering relevant content to state agencies and stakeholders to better manage and support these essential needs, rather than serving as duplicate data repositories.

PR-5. Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary?

Provider Directory API: Surescripts is the co-lead of the FAST National Directory work group[3], which published STU1 of National Directory, and is actively working on an STU2[4] publication. A national provider directory must have strong governance to manage data quality. CMS must prioritize API-based, machine-readable data attestation instead of manual attestation through web portals or flat files.

PR-9 Digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship?

Surescripts suggests IAL2 identity verification as a requirement to access certain nationwide provider directory features. The IAL2 standard is already being adopted for other provider use cases (i.e., The Joint Commission, Electronic Prescribing of Controlled Substances (EPCS)). If CMS required IAL2 for providers seeking to access Direct Primary Care or to verify their National Directory entry, it would allow for reuse of the existing IAL2-verified credential for other use cases, including at the provider's local health system (i.e. account recovery, EPCS). This would drive a better provider experience while improving operational efficiency and reducing fraud across the healthcare ecosystem.

A reusable, networked digital identity enhances patient privacy as well as processing efficiency by removing friction for returning users and tying an identity to unique user biometrics. This ensures each person is uniquely proofed and identified across the network based on who they are rather than what they know (i.e., username and password). As a user, once you have enrolled at any point in the system, you never need to go through that friction again. For example, a patient can reuse the same identity to create an account, reset their password, and check-in at the clinic. Simultaneously, healthcare organizations share the burden of onboarding users across the network so there is no need to re-enroll a user every time they move from one service to another, reducing overall cost, user friction, and increasing operational efficiency.

The core benefits of a networked digital identity for healthcare organizations are summarized as follows:

1. **Convenience and Speed**: A reusable networked identity leverages biometric data to streamline identity verification. Users can prove who they are once at high fidelity (IAL2) and carry that identity across the network, which improves efficiency and reduces fraud.

[3] Home - National Directory of Healthcare Providers & Services (NDH) Implementation Guide v2.0.0
[4] Download Specification - National Directory of Healthcare Providers & Services (NDH) Implementation Guide v2.0.0

2. **Reusable Identity**: A reusable networked identity allows users to create a single, verified digital identity that can be used across various platforms and services. This eliminates redundant identity checks and manual data entry, improving user experience and reducing the risk of drop-offs in onerous processes. CLEAR's 31M+ existing users can verify instantly with a selfie photo, accelerating adoption and conversion on Day 1.

3. **Cost Savings:** A reusable identity means the cost of identity proofing is amortized across the network of providers. Therefore, verified experiences cost a fraction of legacy identity verification platforms (i.e., knowledge-based authentication).

4. **Improved Patient Matching and Reduced Duplicates:** Demographic information (i.e., name, date of birth, address) is verified and corroborated with third party sources, leading to a high patient match rate and reduction in duplicate health records. When a strong identity is paired with a robust master patient index there is high confidence in who the patient is, and that it is the same patient throughout the system.

5. **Identity Orchestration:** Different experiences should command different levels of assurance. For example, a lightweight verification may be appropriate for patient scheduling, whereas a document-based level of assurance is appropriate for account creation or check-in, with IAL2 being reserved for the highest risk use cases such as password reset, Individual Access Services, EPCS, and more. Lightweight verification, for example, would not be sufficient for accessing personal health information (PHI), so it is important the rule sets appropriate standards depending on the sensitivity and security importance of the actions.

6. **Security and Privacy**: The importance of data security and privacy, whether at rest or in transit, is paramount given the sensitive nature of healthcare data. Users need to understand and agree to what data they are sharing. CLEAR is built to protect personal information and keep users in control — allowing users to control what personal information is shared, while maintaining high security standards within the industry, and with NIST and DHS. CLEAR never shares data without explicit user consent. This protects sensitive information and mitigates risks like identity theft.

PR-10. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs):

CLEAR's work with Surescripts is an innovative example of how the private sector has invested in solutions. Surescripts enforces IAL2 across their network - applying digital identity to protect directory integrity and secure healthcare communications. Surescripts has realized significant impact including:

1. 2x Higher Verification Success Rate: 80% with CLEAR vs. 41% with the legacy solution, a major improvement given IAL2's rigorous requirements.

2. 40% Faster Process: CLEAR replaced a lengthy, multistep process with a quick, one-time setup and instant verification with a selfie photo, unlocking faster activation and revenue potential.

3. 50% Boost in Pull-through: More providers are completing onboarding and getting to work faster.

PR-11 *How could members of trust communities (for example, QHINs, participants and sub participants in TEFCA, which requires* ***Identity Assurance Level 2 (IAL2) via Credential Service Providers (CSPs))*** *better support the goals of reduced provider and patient burden while also enhancing identity management and security?*

- **TD-3 Regarding Digital Identity:** *What are the challenges and benefits?*

Surescripts supports the use of technologies that can improve the verification process by making it more secure and less burdensome to users. We believe entities such as CLEAR, Login.gov, ID.me, and other NIST 800–63–3 IAL2/AAL2 certified CSPs can play an important role in this regard, giving providers and payers greater confidence that an individual is who they profess to be, and allowing for a simpler process for patients and caregivers to verify their identity. Organizations such as the Kantara Initiative[5] have played an important role in certifying alignment with the NIST standards.

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers?

We support the idea of a nationwide provider FHIR endpoint directory to ease connections between systems and stakeholders. A centralized directory maintained by a trusted entity, such as CMS or ASTP/ONC, would help all health IT platforms dynamically discover the proper location for sending data or queries for a specific provider or organization. We believe this national directory will also need to support the federation of identities from the three largest credential providers. However, the systems must be set up to recognize the authority of the 'individual' versus the 'employer' organization.

The nationwide directory's primary value will be to have the latest official names of individuals, and where to look up the chain for additional information to answer questions such as the following:

- Which organizations is the provider affiliated with?
- How does one connect with them securely?

We see the directory as the starting point rather than the full solution. Surescripts' provider directory was built to address the use case of an instant search for frequently changing elements plus bulk data to meet local performance requirements.

We offer the following scenario for consideration: if the provider uses a CSP to access and approve their information, that same credential should be re-usable to access other applications in the ecosystem. This will remove friction and eliminate the need for providers to re-enter their details, saving them time and adding efficiency. Pairing this credential with the Unified Data Access Profiles (UDAP), as part of the HL7 Security for Scalable Registration, Authentication, and Authorization (SSRAA), will further accelerate secure access at scale. Surescripts is actively engaged as both contributors to the standards development and has built corresponding reference implementations and software development kits (SDKs).

[5] https://kantarainitiative.org/us-identity-certification-program/

TD-6 What unique interoperability functions does TEFCA perform?
a. What existing alternatives should be considered?
b. Are there redundant standards, protocols or channels or both that should be consolidated?

TEFCA is not particularly unique. Its design is to consolidate the existing frameworks performing national exchange functions and create a simplified framework beyond regional and state HIE services to connect all of healthcare. TEFCA's design is intended to replace the alternatives noted below.

a. Carequality is nearly an exact replica of TEFCA's request and response workflows but should not be considered as a replacement because the framework lacks the clear centralized authority created with the RCE and the oversight by ASTP/ONC. DirectTrust is a better alternative to the TEFCA push messaging because it is more broadly adopted, has a model that supports both individual and organizational approaches to push messaging, and has a stronger validation and trust framework. This should be seen as a superior framework to TEFCA's push model.

b. **Redundant standards:** Carequality to TEFCA for request response workflows. TEFCA to Direct Messaging for push messaging workflows. HIE models should be incorporated and folded into TEFCA. The DEA model for supporting EPCS took the approach of specifying the conditions for exchange - without dictating a standard – allowing the industry to adapt. Additionally, with respect to standards, we suggest CMS and ASTP/ONC avoid creating a "ceiling," which may stifle innovation when entities believe they are constrained to only meet the bar.

TD-9 Regarding certification of health IT:
c. How could ASTP/ONC revise health IT certification criteria to require APIs to consistently support exchanging data from all aspects of the patient's chart (for example, faxed records, free text, discrete data)?

The certification of an API does not fully address the information exchange issue. APIs alone do not ensure an appropriate information exchange process will occur. We suggest the emphasis should be placed less on the API itself or its technical mechanics, and more on defining the goals regarding what information should be shared, the timing of the exchange, the rationale behind sharing the information, and how patient autonomy can be preserved in the decision-making process when appropriate.

TD-11 Regarding EHI export: Now that this capability has been in production for over a year, CMS and ASTP/ONC seek input on the following:
a. Should this capability be revised to specify standardized API requirements for EHI export?
b. Are there specific workflow aspects that could be improved?
c. Should CMS consider policy changes to support this capability's use?

We recommend CMS and ASTP/ONC consider corresponding policy changes to encourage the use of TEFCA to support this type of exchange. Individual patient exchange of health information is already occurring at scale. We suggest that an API requirement involving bulk exchange of health information will drive the RCE and QHINs to accommodate this process and implement new features to support the bulk exchange.

TD-14. Regarding networks' use of FHIR APIs:

Surescripts is a QHIN under TEFCA. In addition, Surescripts connects virtually all healthcare providers, EHRs, Pharmacies, and pharmacy benefit management organizations, through standards like NCPDP, X12, Direct, IHE, HL7 v2, and HL7 FHIR. Over more than two decades of building our network, we have also defined enhancements to standards to meet the industry's needs. Our purpose-built directory includes real-time API integrations to all connected systems for maintaining and retrieving participant records. Surescripts is currently a co-lead of the FAST FHIR National Directory workgroup to define a FHIR standard for the future. This effort builds from our experience working with industry on previous initiatives from CAQH defining An Industry Roadmap for Provider Data, and earlier standardization efforts with ONC (e.g., Healthcare Provider Directory [HPD]).

VB-2 How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

Pharmacists have long played an important role in increasing access to care by providing clinical services as a member of the patient's care team. However, pharmacy practice settings are unique and often disconnected from the rest of the health care team - making care delivery less efficient and complete. Surescripts is proud to be an elite sponsor of the Sequoia Project's Pharmacy Workgroup. This nationwide community of practice launched earlier this year as part of the Sequoia Project's Interoperability Matters Program[6] and includes members from over 45 different healthcare organizations from across the industry. The Workgroup is focused on finding solutions that enable pharmacists to provide clinical services at scale, including pharmacy participation in TEFCA. The Workgroup aims to close gaps in pharmacy interoperability by identifying and operationalizing information that pharmacists need to access and share with other providers and payers.

Integrating pharmacists into APMs broadens the network of accessible care providers to reach underserved areas and patients with complex medication needs. Pharmacist interventions can reduce hospital readmissions, lower overall healthcare costs, and improve population health management. Measures that advance interoperability are needed to further enable pharmacists to improve quality ratings, patient satisfaction, and overall health outcomes.

Based on a recent survey conducted by the National Association of Chain Drug Stores (NACDS), the majority of NACDS members who responded to the survey have participated in some type of value-based care arrangement, including a mix of pay-for-performance, risk-based agreements, and shared savings models. The most common clinical interventions part of value-based care models that NACDS members have participated in include medication adherence initiatives, gap closures, A1c and blood pressure measurement and improvement initiatives, and vaccinations, in alignment with CMS star ratings measures, and HEDIS measures.

VB-3. What are essential health IT capabilities for value-based care arrangements?

[6] Pharmacy Workgroup - The Sequoia Project

We strongly recommend CMS and ASTP/ONC review and implement the findings and recommendations from the *Final Report of the Health Information Technology Advisory Committee on Pharmacy Interoperability and Emerging Therapeutics*[7].

We call out the following specific charge: Identify opportunities and recommendations to improve interoperability between pharmacy constituents (prescribers, pharmacists, pharmacy benefit managers, dispensers, payers, intermediaries, PDMPs, public health agencies, HIEs, third party service providers, consumers, etc.) for pharmacy-based clinical services and care coordination.

Surescripts appreciates the opportunity to provide feedback on opportunities for creating a stronger, more resilient, and interoperable digital health ecosystem. We look forward to continuing to work with CMS and ASTP/ONC on implementing these recommendations and building upon innovative technologies and processes. We are happy to be resource for your teams and I may be contacted at deanne.primozic@surescripts.com.

Sincerely,

Deanne Primozic

Deanne Primozic, M.S.
Vice President, Policy and Federal Affairs

[7] Pharmacy Interoperability and Emerging Therapeutics Task Force 2023 Recommendations Transmittal Letter November 9, 2023